

ACUGRAIN TABLET 0.5MG

DESCRIPTION: A 7mm round, scored tablet, yellow in colour with markings 'DUOMA/DUOMA' & 'DUO/2'

COMPOSITION:

Each tablet contains Pizotifen Malate equivalent to Pizotifen 0.5 mg.

PHARMACODYNAMICS:

Studies in experimental animals have indicated that pizotifen has strong anti-serotonin and anti-tryptaminic properties, marked antihistaminic effects and some antagonistic activity against kinins. It also possesses weak anti-cholinergic effects and sedative properties. Pizotifen also possesses appetite-stimulating properties.

The prophylactic effect of pizotifen in migraine is associated with its ability to modify the humoral mechanisms of headache.

It inhibits the permeability-increasing effect of serotonin and histamine on the affected cranial vessels, thereby checking the transudation of plasmakinin so that the pain threshold of the receptors is maintained at 'normal' levels. In the sequence of events leading to migraine attack, depletion of plasma serotonin contributes to loss of tone in the extracranial vessels. Pizotifen inhibits serotonin re-uptake by the platelets, thus maintaining plasma serotonin and preventing the loss of tone and passive distension of the extracranial arteries.

PHARMACOKINETICS:

Absorption: The absorption of pizotifen in man is fast (absorption half-life 0.5 to 0.8 hours) and nearly complete. The absolute bioavailability is 78%. Maximum blood levels are reached 5 hours after a single 2 mg oral administration of pizotifen (parent compound and N-glucuronide-conjugate measured together).

Distribution: Protein binding of pizotifen in human plasma in vitro amounts to 91%. The distribution volume in man is 833 L and 70 L for pizotifen and its N-glucuronide, respectively.

Metabolism: Pizotifen is extensively metabolized. Glucuronidation is the main route of biotransformation and the main metabolite is the N-glucuronide-conjugate, accounting for at least 50% of the plasma and 60-70% of urinary excreted radioactivity.

Elimination: About one-third of an orally applied dose is excreted via the biliary route into the faeces, a significant proportion, corresponding to about 18% of the applied dose, representing parent drug, likely produced in the intestine after biliary excretion of the N-glucuronide-conjugate. Less than 1% of the administered dose of pizotifen is excreted unchanged in the urine, whereas up to 55% is excreted as metabolites. Pizotifen is eliminated with a half-life of approximately 23 hours (total radioactivity).

Unchanged pizotifen and the N-glucuronide have, as estimated from the excretion in the urine, a comparable elimination half-life.

INDICATION:

Prophylactic (interval) treatment of vascular headaches; typical and atypical migraine; vasomotor headache, cluster headache (Horton's syndrome).

It is not effective in relieving migraine attacks once in progress.

Pizotifen is less effective in tension headaches and in psychogenic and post-traumatic headaches.

RECOMMENDED DOSAGE:

Adult: The dosage should be progressively increased, starting from 0.5 mg daily. The average maintenance dosage is 1.5 mg daily in divided doses or as a single dose at night. In refractory cases the dosage may be usually raised to 3-4.5 mg daily in two or three divided doses.

Children: (aged over 2 years): Up to 1.5 mg daily, usually as a divided dose, although up to 1 mg has been given as a single dose at night.

In patients with kidney insufficiency, dosage adjustment may be necessary.

ROUTE OF ADMINISTRATION: Oral

CONTRACINDICATIONS:

Hypersensitivity to the drug or to any of the other tablet ingredients. Pizotifen should not be given to children under 2 years of age.

WARNINGS & PRECAUTIONS:

Patients should be warned that, owing to its possible sedative effect, pizotifen may slow their reactions when driving vehicles or operating machinery.

Although the anticholinergic activity of Pizotifen is relatively weak, caution is required in the presence of closed-angle glaucoma and in patients with a predisposition to urinary retention. Dosage adjustment may be required in patients with renal insufficiency.

Pizotifen should be used with caution in patients with a history of epilepsy.

Pizotifen tablets contain lactose. Patients with rare hereditary problems of galactose intolerance, severe lactase deficiency or glucose-galactose malabsorption should not take Pizotifen tablets.

EFFECT ON ABILITY TO DRIVE AND USE MACHINES

Pizotifen may cause drowsiness, somnolence and dizziness. Therefore, caution should be exercised when driving or using machines.

Patients being treated with Pizotifen and presenting with drowsiness (including somnolence and fatigue) must be instructed to refrain from driving or engaging in activities where impaired alertness may put themselves or others at risk.

INTERACTION WITH OTHER MEDICAMENTS:

The central effects of sedatives, hypnotics, antihistamines (including certain common cold preparations) and alcohol may be enhanced by pizotifen.

Pizotifen antagonises the hypotensive effect of adrenergic neurone blockers.

USE IN PREGNANCY & LACTATION:

Use in pregnancy: As clinical data with pizotifen in pregnancy are very limited, it should only be administered under compelling circumstances.

Use in lactation: Use in nursing mothers is not recommended.

SIDE EFFECTS:

The most commonly reported side-effects are appetite stimulating effect, increase in body weight and drowsiness (including somnolence and fatigue).

Immune system disorders	
Rare	Hypersensitivity reactions, face oedema, urticaria and rash
Metabolism and nutrition disorders	
Very common	Appetite stimulating effect and increase in body weight
Psychiatric disorders	
Rare	Depression, CNS stimulation (e.g. aggression, agitation), hallucination, insomnia, anxiety
Nervous system disorders	

<i>Common</i>	Drowsiness (including somnolence) dizziness
<i>Rare:</i>	Paraesthesia
<i>Very rare:</i>	Seizures
Gastrointestinal disorders	
<i>Common</i>	Nausea, dry mouth
<i>Uncommon</i>	Constipation
Musculoskeletal and connective tissue disorders	
<i>Rare</i>	Myalgia, arthralgia
General disorders and administration site conditions	
<i>Common</i>	Fatigue

Acute withdrawal reactions have been reported following abrupt cessation of Pizotifen therefore gradual withdrawal is recommended. Withdrawal symptoms include anxiety, tremors, insomnia, nausea and loss of consciousness.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptoms: Drowsiness, dizziness, hypotension, dryness of the mouth, confusion, excitatory states (in children), ataxia, nausea, vomiting, dyspnoea, cyanosis, tachycardia, convulsions (particularly in children), coma, and respiratory paralysis.

Treatment: Administration of activated charcoal is recommended; in case of very recent uptake, gastric lavage may be considered. Severe hypotension must be corrected (CAVE: adrenaline may produce paradoxical effects). If necessary, symptomatic treatment including monitoring of the cardiovascular and respiratory systems. Excitatory states or convulsions may be treated with short acting benzodiazepines.

STORAGE CONDITIONS: Store below 30°C. Protect from light. Keep out of reach of children. Jauhkan daripada kanak-kanak.

SHELF LIFE: Please refer to outer package.

PACK SIZE: Blister pack of 3 x 10's and, 30 x 10's in a box. (Note : All presentations may not be available in all markets).

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD
Lot 2599, Jalan Seruling 59 Kawasan 3,
Taman Klang Jaya, 41200 Klang, Selangor, MALAYSIA.